

REXOMTM MIST. MAGNESIUM TRISILICATE

Composition and Strength of Active Ingredients:

Each 5 ml contains :

Magnesium Trisilicate	250mg
Light Magnesium Carbonate	250mg
Sodium Bicarbonate	250mg

Preservatives:

Sodium Methyl Paraben	0.14% w/v
Sodium Propyl Paraben	0.08% w/v
Cetylpyridinium Chloride	0.1% w/v

Product Description

White suspension with peppermint flavour.

Pharmacodynamics

Magnesium trisilicate is a hydrated magnesium silicate. It is an antacid with general properties like of magnesium hydroxide. Magnesium trisilicate is also used as a food additive and as a pharmaceutical excipient.

Magnesium carbonate serves as an antacid to neutralise stomach acidity by forming soluble magnesium chloride and carbon dioxide with longer lasting effect due to its slow onset of action.

Sodium bicarbonate is a short acting antacid which neutralizes the acidity in the gastrointestinal tract by reacting with hydrochloric acid to produce sodium chloride. During neutralisation, carbon dioxide is released to provide a sense of relief.

Pharmacokinetics

When given orally it reacts more slowly with hydrochloric acid in the stomach than magnesium hydroxide. Magnesium chloride and hydrated silica gel are formed during the neutralisation. About 5% of magnesium is absorbed and traces of liberated silica may be absorbed and excreted in the urine.

Neutralisation of acidity in stomach by magnesium carbonate produces soluble magnesium chloride and carbon dioxide in the stomach whereby magnesium is absorbed and excreted rapidly via renal in the urine.

Sodium bicarbonate neutralises gastric acid with the production of carbon dioxide whereby bicarbonate ions that are not involved in the reaction is absorbed. In the absence of bicarbonate insufficiency in plasma, the ions are excreted in the urine which is depicted as alkaline followed by diuresis.

Indications

It is indicated for symptomatic relief of stomach upset associated with hyperacidity (heartburn, acid indigestion, and sour stomach); hyperacidity associated with gastric and duodenal ulcers and in the management of reflux esophagitis.

Recommended Dose

Adult and children over 12 years: 10-20ml 3 times daily or as directed by physician.
Child 5-12 years: 5-10 ml 3 times daily or as directed by physician.
Take in a little water.

Route of Administration

Oral

Contraindications

Generally, contraindicated in patients that are hypersensitive to antacids or with hypophosphataemia.

Magnesium carbonate is contraindicated in patients with heart block and severe renal impairment.

Sodium bicarbonate is contraindicated in patients with controlled sodium intake, heart failure, hypertension, renal failure, cirrhosis, diarrhoea.

Warning and Precautions

Contraindicated in severe renal failure, hypophosphataemia and in patients who must control sodium intake, should not be administered to patients with metabolic

or respiratory alkalosis, hypocalcaemia or hypochlorhydria and hypersensitivity to any of the ingredients.

Magnesium carbonate should be cautiously given to patients with less severe degree of cardiac or renal impairment and should be monitored for clinical signs of excess magnesium.

Sodium bicarbonate should be given extremely cautiously to patients with metabolic or respiratory alkalosis, hypocalcemia, hypochlorhydria, heart failure, oedema, renal impairment, hypertension, eclampsia or aldosteronism.

Interactions with Other Medicaments

Generally, antacids may interact with other drugs by altering their absorption and sometimes elimination. Antacids may also damage enteric coatings designed to prevent dissolution in the stomach.

Oral magnesium salts can possibly decrease the absorption of tetracyclines and bisphosphonates and doses should be separated by a number of hours.

Sodium bicarbonate enhances the lithium excretion and increased renal clearance of acidic drugs such as salicylates, tetracyclines and barbiturates. Sympathomimetics and stimulants prolongs the half-life of basic drugs enhancing toxicity.

Pregnancy & Lactation

There are no sufficient studies among pregnant women and therefore it is advisable to use with caution under the medical advice of doctor.

Magnesium carbonate is not recommended during pregnancy unless fetal heart rate is monitored and usage within 2 hours of delivery should be avoided as magnesium is reported to cross the placenta.

No clinical data on pregnancy nor neonates are reported for sodium bicarbonate. However, it is advisable to cautiously prescribe to pregnant women.

Side Effects

Magnesium salts may cause diarrhoea in some patients.

Magnesium carbonate may cause diarrhoea, hypermagnesia, manifesting symptoms like nausea, vomiting, flushing of skin, thirst, hypotension, drowsiness, confusion, muscle weakness.

Sodium bicarbonate can cause stomach cramps, belching and flatulence.

Symptoms and Treatment of Overdose

Symptoms of overdose are high possibilities of hypermagnesia, hypernatremia and hyperphosphatemia.

Treatment of overdose to give calcium salts via intravenous administration for hypermagnesia, to give plenty of salt free liquids for the treatment of hyponatremia.

Effects on Ability to Drive and Use Machine

N/A

Storage Condition

Keep the container tightly closed. Store below 30°C. Protect from heat and light.

Dosage Forms and Packaging Available

60 ml, 90 ml, 100 ml, 120 ml, 200 ml and 240 ml bottle.
120 ml and 200 ml HDPE Bottle (For Brunei only)
3.6L per container (For export only or KKM tender only).

Registration Number:

MAL19962486X

Manufactured by & Product Registration Holder:

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Date of Revision:

19 December 2024

PI0001-4